ONE Clinical Respiratory Protocol

POSSIBLE F-TAG/QMS

F Tag 695

- Respiratory/Tracheostomy Care and Suctioning

F Tag 636

- Comprehensive Assessments & Timing

F Tag 637

- Comprehensive Assessment After Significant Change
- Potentially preventable 30 day post discharge readmissions – Overall return to acute and hospital readmissions.

THERAPIES

- PT
- OT
- SLP

PURPOSE OF THERAPY

- Exercise prescriptions – aerobic conditioning, resistance training, flexibility exercises
- Breathing Exercises to improve peak work capacity
- Balance and Gait training
- Postural management for pain management and respiratory ease – UE positioning with shoulders in adduction, extension and internal rotation will facilitate diaphragmatic breathing
- Positioning for adequate respiration at rest and with activity – lumbar rolls to promote anterior pelvic tilt
- Facilitate conversational speech
- Facilitate sustained phonation
- Improve coordination of breathing with speech and/or swallowing
- Improve Cough Strength for airway protection

GOAL EXAMPLES

PT/OT:

- Pt. will complete breath support exercises (name specific exercise) with (% accuracy) to sustain normal volume during conversational speech.
- Pt. will increase maximum phonation time to greater than 15 seconds to improve breath support for speech at a conversational level.
- Pt. will demonstrate increased O2 saturation from 93% to greater than 95% following completion of (name of specific) breathing strategy.
- Pt. will use pacing strategies during conversational speech with caregivers with (%) accuracy.
- Patient will utilize necessary adaptive equipment to safely complete UB/LB dressing tasks with minimal assistance while maintaining oxygen saturation levels > 90% with minimal verbal cues for breathing techniques.
- Patient will independently perform breathing technique to maintain oxygen saturation levels above 90% and to minimize SOB for improved carryover during ADL, IADL, and functional mobility

TOOLS & ASSESSMENTS

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- ACP Plus Respiratory Assessment (ARA) – measure forced inspiratory volume, forced expiratory volume, forced vital capacity
  - Provide obstructive and restrictive interventions accordingly – forced expiration, rhythmical breathing, cough Omni-Flow
- Incentive Spirometry
- Diaphragmatic and pursed lip breathing Manual chest wall mobilization
- Peak Flow Meter
- Assessment of Posture
- Modified BORG Dyspnea Scale
- MOCA or SLUMS to determine baseline cognitive functioning
- Vital Signs – pulse oximetry, respiratory rate, blood pressure, heart rate
- 6 minute walk test
- 2 min step test
- BERG
- Modified Fatigue Impact Scale
- COPD Self Efficacy Scale
- Chair Rise Test
- Canadian Occupational Performance Measure (COPM)
- E stim for improving muscle performance and exercise tolerance
- Vital Signs

POSSIBLE F-TAG/QMS	THERAPIES	PURPOSE OF THERAPY	GOAL EXAMPLES	TOOLS & ASSESSMENTS
		Improve Vocal Quality and/or Loudness Swallow Interventions Cognitive interventions strategies related to respiratory function, use of respiratory equipment or management of disease Assessment of ADLs and impact respiratory diagnosis has on performance Energy conservation and work simplification training Psychosocial adaptations Relaxation Techniques Modalities	Patient will demonstrate stress reduction strategies with minimal cues, to reduce anxiety and decrease respiratory rate with 100% carryover. Patient will reduce RPE to less than 12/20 post seated showering task with occasional verbal cues for breathing techniques and energy conservation/work simplification. Patient will complete self-care tasks within 30 minutes, with less than 2 rest breaks, while maintaining oxygen saturation levels >90% on room air. Patient will have a decreased RPE (10-11) with household ambulation distances with Supervision to allow mobility without evidence of respiratory distress. Patient's six-minute walk test will improve to _____ score to allow for improved activity tolerance with facility mobility tasks. **SLP:** Patient will improve speech intelligibility for phrases to 90% to improve communication of needs Patient will demonstrate functional voice quality as measured by VHI during short conversations to improve ease of communication Patient will use RMST to facilitate improved vocal loudness during communication of needs via improved respiratory strength as measured by RMST device and MPT	**Motor Speech Assessment** Cranial Nerve Newcastle Motor Speech Tifosky word list Rainbow or Grandfather Passage Colorado Motor Speech Framework **Voice** GRBAS VHI CAPE-V S/Z ratio MIP/MEP if assessing for RMST or use device to obtain starting point **Respiratory related approaches for Speech/Voice** RMST Diaphragmatic breathing Respiratory-speech coordination LSVT LOUD or SPEAK OUT! Positioning Voice Relaxation **Cognition** Neuro-Qol PROMIS Cognitive Function Modified Fatigue Impact Scale Multifactorial Memory Questionnaire Communicative Participation Item Bank

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			Patient will state 3 scenarios where they have increased oxygen needs to improve resp. function during daily tasks. (cog) Patient will improve cough strength via RMST to decrease risk of airway compromise during PO intake of thin liquids Will demonstrate adequate coordination of breathing and swallowing on 8/10 therapeutic PO intake trials in order to improve safety and reduce risk of respiratory compromise. Patient will report decreased modified Borg scale score(< 3) during PO intake in order to improve endurance for safe and efficient swallow function. Patient will adequately manage secretions during therapy session, as evidenced by no need for suction during 30 min session to improve swallow function for secretion management.	La Trobe Communication Questionnaire Iowa Trail Making Test Stroop Test Community Integration **Measure** Hopkins Verbal Learning Royal Prince Alfred Prospective Memory Test Baseline general cognitive function **Respiratory Related approaches for Cognition** Meta-cognitive strategy to manage respiratory needs Memory strategies and external memory aids to recall respiratory needs Sequencing for use of respiratory equipment Problem solving related to respiratory needs **Swallowing** MASA Eat-10 PEFr-peak expiratory flow rate for cough 3 ounce Water test Test of Masticating and Swallowing Solids(TOMASS) Cranial Nerve Exam FEEs if needed MEP/MIP for Respiratory baseline if using RMST or use of device for baseline **Respiratory related approaches for dysphagia** RMST-For Swallow and respiratory muscle strengths VCST-voluntary cough skill training Breathing- Swallow Coordination Strategies such as Supra-glottic swallow